

KORVIN MERROW WORLD SPECIFICATION DOCUMENT

World Title: Korvin Merrow | World Type: Typical Clinical World | Snapshot: HD6 05/23/2026 18:00
Clinical domain: acute hospital medicine, medication reconciliation, consultant synthesis, discharge planning

1. Clinical Scenario

1.1 Big Picture Summary

Korvin Merrow is a 62-year-old man admitted after about three weeks of progressive weakness, poor oral intake, lightheadedness with a near-fall, family-noticed confusion, possible urinary symptoms, and medication-management mistakes. He has common but high-stakes chronic disease: HFrEF, CAD with remote PCI/stent, CKD stage 3, type 2 diabetes, PMR with chronic prednisone exposure, neuropathy, OSA, anemia of CKD, steroid-related bone disease, and polypharmacy.

The initial ED and admission frame is suspected urinary-source infection with dehydration/volume vulnerability, altered baseline mental status, functional decline, and AKI-on-CKD risk. Sepsis-oriented management is clinically reasonable. The hard part is that partial improvement does not make the patient safely solved. By HD6 he looks better, but discharge safety depends on reconciling renal and cardiac medication timing, prednisone-source reliability, functional recovery, family concerns, consultant recommendations, and home support.

The world closes on 05/23/2026 at 18:00 during discharge planning. The downstream tasks ask for physician-facing work that a careful hospital clinician, primary care clinician, transition team, or medical director could reasonably produce: medication reconciliation, discharge summary, care coordination, consultant synthesis, early follow-up assessment, and safety/readmission-risk review.

1.2 Patient Profile

Field	Value
Patient name	Korvin Merrow
World Type	Typical Clinical World
DOB / age	1964-02-18 / 62 years
MRN	KM-6427819
Height / weight / BMI	178 cm (5'10") / 97 kg (214 lb) / BMI 30.6
Allergy / intolerance	Lisinopril (cough), treated as ACE-inhibitor intolerance
Code status	Full Code
Baseline living situation	Lives with family; family knows his usual function and medication routines
Baseline function and cognition	Independent with basic activities but slowed by chronic illness; occasional cane use; mild age-related forgetfulness only
Baseline clinical anchors	Creatinine 1.6-1.8 mg/dL; eGFR about 40-50 mL/min/1.73 m2; hemoglobin 10.5-11.5 g/dL; A1c 7.6-8.2%; dry weight about 97 kg

Korvin lives with family and was independent with basic activities before the decline, though slowed by chronic illness. He used a cane occasionally, had only mild age-related forgetfulness, and could participate in medication routines with family oversight. His usual function matters because the hospitalization produces improvement without a clean return to baseline.

Baseline clinical anchors are comparators, not admission or hospital-course values. They should help clinicians distinguish chronic CKD, chronic anemia, diabetes control, dry weight, baseline mobility, baseline cognition, and usual medication-management ability from acute illness and discharge-readiness concerns.

Baseline / reported home medication list

#	Medication	Dose / strength	Route	Frequency	Indication / clinical role
1	Aspirin	81 mg	PO	Daily	CAD secondary prevention
2	Atorvastatin	40 mg	PO	Nightly	CAD / hyperlipidemia
3	Sacubitril/valsartan	24/26 mg	PO	BID	HFrEF guideline-directed therapy after ACE-inhibitor intolerance
4	Carvedilol	12.5 mg	PO	BID	HFrEF / CAD; hemodynamic-sensitive therapy
5	Furosemide	40 mg	PO	Daily	HFrEF volume management
6	Spirolactone	25 mg	PO	Daily	HFrEF mineralocorticoid

#	Medication	Dose / strength	Route	Frequency	Indication / clinical role
					antagonist; renal and potassium safety relevant
7	Empagliflozin	10 mg	PO	Daily	HFrEF / type 2 diabetes; acute-illness hold and restart reasoning
8	Metformin ER	500 mg	PO	BID	Type 2 diabetes; renal and acute-illness safety relevant
9	Insulin glargine	18 units	SC	Nightly	Type 2 diabetes basal insulin
10	Gabapentin	300 mg	PO	Nightly	Diabetic neuropathy; functional/cognitive safety context
11	Ferrous sulfate	325 mg	PO	Every other day	Anemia of CKD; constipation burden relevant
12	Alendronate	70 mg	PO	Weekly	Steroid-related osteoporosis/osteopenia
13	Prednisone	No fixed dose assigned	PO	Reported tapering schedule unclear	PMR; recent taper/dose varies across chart sources
14	Calcium carbonate / vitamin D	600 mg / 400 IU	PO	BID	Bone health support with chronic steroid exposure
15	Nitroglycerin	0.4 mg	SL	PRN chest pain	CAD angina rescue medication
16	Acetaminophen	650 mg	PO	q6h PRN	Pain / musculoskeletal symptom support
17	Pantoprazole	40 mg	PO	Daily	GERD / acid-suppression indication
18	PEG 3350	17 g	PO	Daily PRN	Constipation tendency / bowel regimen support
19	Senna	8.6 mg	PO	Nightly PRN	Constipation tendency / bowel regimen support

Prednisone note

Prednisone remains deliberately non-fixed in the World Spec. Chart sources disagree about the recent taper/dose and adherence. The task is to reconstruct risk and safe handling from source hierarchy, not to assume a single hidden dose. A separate vitamin D-only row is not included because vitamin D is already carried in the calcium carbonate / vitamin D combination.

1.3 Clinical History and Context

Before admission, Korvin was chronically ill but functioning at home. Around 04/27/2026, he began a gradual decline marked by reduced stamina, less activity, poor appetite, reduced fluid intake, increasing weakness, more dependence on family, possible urinary symptoms, progressive unsteadiness, cognitive slowing compared with baseline, and medication-management mistakes. The near-fall is multi-factorial rather than a single-cause event: poor intake, volume depletion, medication effects, neuropathy, deconditioning, infection physiology, and steroid-related physiology can all contribute.

On HD1, ED and admission teams reasonably prioritized suspected urinary-source infection, dehydration or volume vulnerability, altered baseline mental status, functional decline, and AKI-on-CKD risk. Early treatment and stabilization made him less acutely ill, but the case remained broader than infection alone. Chronic cardiorenal medications, diabetes therapy, prednisone exposure, and baseline functional reserve all became relevant as the hospitalization evolved.

By HD3 and HD4, important information came from outside the easiest physician-summary narrative. Nursing, PT, OT, and family observations described incomplete functional and cognitive recovery. Cardiology, Nephrology, and Endocrinology each had defensible concerns, but their recommendations depended on timing, clinical trends, medication actions, and what problem they were prioritizing. Rheumatology and pharmacy provenance helped clarify prednisone intent while still leaving recent actual use imperfectly reconstructed.

By HD5 and HD6, Korvin was medically improved enough for discharge planning to be plausible. That improvement did not erase the practical risks. The plan still had to account for renal recovery, potassium and blood pressure reserve,

HFrEF/CAD medication protection, diabetes therapy, prednisone instructions, therapy and nursing assessments, family support, education, follow-up, and whether a visible discharge-facing plan was complete enough to trust.

Comorbidity profile

#	Condition	Clinical role
1	Heart failure with reduced ejection fraction	Creates long-term cardioprotective therapy and cardiorenal medication timing tension.
2	Coronary artery disease with remote PCI/stent	Supports secondary prevention, cardiology involvement, and risk if protective therapy is lost.
3	Hypertension	Fits CKD/CAD/HFrEF and hemodynamic caution during acute illness.
4	Hyperlipidemia	Supports chronic CAD risk reduction and medication continuity.
5	Chronic kidney disease stage 3	Makes AKI recovery, renal-sensitive medications, and nephrology input clinically important.
6	Type 2 diabetes mellitus	Supports infection vulnerability, medication complexity, renal medication safety, and neuropathy.
7	Diabetic peripheral neuropathy	Contributes to baseline mobility vulnerability and the multi-factorial near-fall.
8	Obstructive sleep apnea	Adds chronic reserve burden without explaining the acute presentation by itself.
9	Polymyalgia rheumatica	Provides chronic prednisone exposure and taper-history uncertainty.
10	Anemia of CKD	Adds fatigue and reduced reserve without becoming an acute anemia story.
11	Osteoporosis / osteopenia from chronic steroid exposure	Corroborates cumulative steroid exposure without proving current adrenal suppression.
12	Class I obesity by BMI	Fits OSA, cardiometabolic disease, mobility reserve, and discharge planning.
13	Chronic GERD / acid-suppression indication	Supports pantoprazole and chronic supportive-medication burden.
14	Chronic constipation tendency	Fits iron, reduced mobility, neuropathy medication, and bowel-regimen complexity.

Procedural history is intentionally quiet: remote PCI with coronary stent placement and remote diagnostic sleep study confirming OSA. There is no ICD, CRT, pacemaker, CABG, dialysis access, major orthopedic repair, amputation, temporal artery biopsy, or major rheumatologic procedure in this version.

Recurring provider roles include Dr. Elian Vossmere (attending hospitalist), Dr. Maris Caldrane (Cardiology), Dr. Iven Solthar (Nephrology), Dr. Nerea Veylorn (Endocrinology), Dr. Talia Quenor (Primary Care), Dr. Soren Halvek (outpatient Rheumatology), and Mara Merrow (family/caregiver). Nursing, PT, OT, Case Management, Social Work, Pharmacy, and rotating hospitalist clinicians contribute role-based evidence.

1.4 Key Milestones

Date / anchor	Event / Anchor
04/27/2026	Approximate beginning of 3-week decline with reduced stamina, activity, appetite, fluid intake, and increasing family dependence.
05/11/2026	Final pre-admission week begins; weakness, unsteadiness, possible urinary symptoms, cognitive slowing, and medication mistakes become more concerning.
05/17/2026	Near-fall/lightheadedness and family recognition of meaningful deviation from baseline drive decision to seek care.
05/18/2026 / HD1	ED presentation and admission. Suspected urinary-source sepsis, dehydration/volume risk, altered baseline mental status, functional decline, and AKI-on-CKD risk are addressed.
05/19/2026 / HD2	Partial improvement after stabilization. Consultant involvement begins; medication safety and steroid history become harder to ignore.
05/20/2026 / HD3	PT/OT and bedside observations make functional and cognitive recovery central to discharge readiness.
05/21/2026 / HD4	Consultant tensions peak. Prednisone history and outpatient provenance become clinically important but not perfectly aligned.
05/22/2026 / HD5	Medical improvement continues; disposition and transition planning become dominant, with medication and support questions unresolved.
05/23/2026 / HD6	World-close day. Patient appears medically improved while discharge safety remains dependent on synthesis across chart sources.
05/23/2026 18:00	World snapshot closes. World-level files stop at or before this timestamp.

Date / anchor	Event / Anchor
05/24/2026	Discharge anchor for task context.
05/31/2026	+7 anchor measured from discharge, used for early follow-up review.
06/23/2026	+30 anchor measured from discharge, used for retrospective safety/readmission-risk review.

1.5 Clinical Complexity Overview

The difficulty comes from realistic hospital medicine rather than a rare diagnosis. A correct response has to preserve mixed physiology: infection may be real and improving, but CKD/HF interaction, steroid exposure, medication effects, neuropathy, anemia, OSA, deconditioning, poor intake, and family-observed functional decline all remain clinically meaningful.

Friction point	Topic	Clinical balance
Cardiology vs Nephrology	Medication restart timing	Cardiology is concerned about preserving HFrEF/CAD protection; Nephrology is concerned about renal recovery, potassium, blood pressure reserve, intake, and volume status. Both positions are clinically defensible.
Family vs Primary Team	Discharge readiness	Family has baseline knowledge and sees persistent vulnerability. The primary team sees objective improvement and plausible discharge planning. The correct pathway integrates both.
Endocrinology vs Primary Team	Steroid interpretation and risk	Endocrinology treats chronic steroid exposure and taper uncertainty as clinically relevant; the primary team is cautious about over-attributing improvement-lag to steroids after sepsis-oriented treatment helps.

Reliability domain	Ordering / rule
Authority hierarchy	Attending hospitalist; consulting attending specialists; PT/OT functional assessments; Case Management/Social Work; family reports; patient recollection.
Clinical fact hierarchy	Attending documentation; verified medication reconciliation; pharmacy history; consultant documentation; primary care documentation; family report; patient recollection.
Prednisone hierarchy	Rheumatology attending recommendation; verified medication reconciliation; pharmacy/refill history; family report; patient recollection.

The key judgment problem is not whether one service, one source, or one document is automatically correct. The clinician must weigh when each source was written, what evidence it used, what it can and cannot answer, and how the patient is actually trending at the discharge-planning snapshot.

2. Task Specifications

All six tasks are physician-facing or physician-supervised clinical deliverables. Supporting sources may come from pharmacy, nursing, PT/OT, case management, social work, family, or other roles, but the final deliverable remains written by or for a physician-level reviewer.

Task 1: Discharge Medication Reconciliation / Medication Safety Review

Workflow: Discharge Medication Reconciliation by the hospitalist/discharge clinician with pharmacy support.

Draft Prompt
I am finalizing Mr. Merrow's discharge medications. Please review the chart and recommend what should be continued, held, stopped, or clarified before discharge, including monitoring and follow-up considerations.

Expected Output: A physician-facing medication safety recommendation that separates chronic CAD prevention, HFrEF/cardiorenal therapy, diabetes therapy, prednisone handling, supportive medications, and home medication-management safety. The response should explain sequencing and monitoring rather than copying a medication list.

Failure Design

Key Trap	Remediation
Medication list copying	Use med rec, pharmacy history, MAR actions, trends, and consultant notes together; do not copy the visible discharge plan as the final medication plan.
Premature or delayed HF/AKI restart	Balance renal recovery, potassium, blood pressure reserve, intake,

Key Trap	Remediation
	volume status, and cardiovascular protection; explain sequencing and monitoring.
Prednisone source error	Use rheumatology as the highest outpatient taper source while integrating med rec, pharmacy, family, and patient recollection; keep the dose/taper uncertainty explicit.
Inpatient-only therapy drift	Do not convert inpatient correctional insulin actions into outpatient medications without independent discharge justification.

Task-level files: FI-T01 and FI-T07. World-level files only otherwise.

Task 2: Hospital Discharge Summary Generation

Workflow: Attending hospitalist discharge summary for a multi-day inpatient course.

Draft Prompt

Please draft a concise discharge summary for Mr. Merrow that accurately explains his presentation, hospital course, consultant input, medication issues, functional status, and follow-up needs.

Expected Output: A discharge summary that tells the clinical evolution from ED presentation through discharge planning, including suspected infection, mixed physiology, consultant chronology, medication reassessment, functional status, family concerns, and unresolved follow-up needs.

Failure Design

Key Trap	Remediation
Copy-forward diagnosis closure	Preserve that the initial infection frame was reasonable while showing why later weakness, medication decisions, steroid history, and disposition issues still mattered.
Consultant chronology flattening	Summarize specialist input by timing and clinical context rather than presenting all recommendations as equally current or final.
Functional evidence omission	Include nursing, PT/OT, and family observations when describing discharge readiness and follow-up needs.
Discharge snapshot overuse	Use the discharge-facing snapshot as a source to reconcile, not as the completed summary.

Task-level files: FI-T02. World-level files only otherwise.

Task 3: Discharge Readiness / Care Coordination Documentation

Workflow: Physician-supervised discharge planning and care-coordination assessment.

Draft Prompt

Please assess whether Mr. Merrow's discharge plan is practical and safe from a care-coordination standpoint, considering function, cognition, home support, services, medication management, and family concerns.

Expected Output: A care-coordination assessment that distinguishes medical improvement from practical readiness and identifies what must be resolved for a coherent transition plan.

Failure Design

Key Trap	Remediation
Medical stability equals readiness	Reconcile improved infection, renal, intake, and mental-status trajectory with therapy, nursing, family, medication-management, and support evidence.
Family concern dismissal	Treat family observations as baseline-informed collateral rather than generic resistance.
Service-plan overconfidence	Recognize that equipment, home health, education, transportation, and caregiver capacity must be confirmed rather than assumed.
Visible plan shortcut	Do not treat the discharge-facing snapshot as proof that all transition requirements have been completed.

Task-level files: FI-T03. World-level files only otherwise.

Task 4: Consultant Synthesis / Interdisciplinary Care Plan

Workflow: Hospitalist-led interdisciplinary care plan integrating specialist, therapy, pharmacy, family, and discharge-planning input.

Draft Prompt

Please synthesize the cardiology, nephrology, endocrinology, hospitalist, therapy, family, and discharge-planning inputs into a physician-facing interdisciplinary plan.

Expected Output: A hospitalist synthesis that explains competing recommendations by issue, preserves reasonable disagreement, and identifies medication, steroid, functional, family, and follow-up items that require coordinated action.

Failure Design

Key Trap	Remediation
Consultant winner selection	Do not automatically defer to Cardiology, Nephrology, Endocrinology, the latest note, or the highest authority; synthesize timing, trends, and patient status.
Cardiorenal oversimplification	Keep both cardiovascular protection and renal/hemodynamic caution clinically valid, then define a monitored sequencing approach.
Steroid overcall	Recognize endocrine concern without making adrenal insufficiency the hidden explanation for the hospitalization.
Disposition siloing	Integrate family, nursing, PT/OT, case management, and medication education with the medical plan.

Task-level files: FI-T04. World-level files only otherwise.

Task 5: Early Post-Discharge Follow-Up Assessment

Workflow: Primary care or transition-team physician review at the +7 discharge anchor.

Draft Prompt

At the one-week follow-up anchor, please identify what needs to be checked based on Mr. Merrow's hospitalization and discharge plan, without assuming any new post-discharge findings.

Expected Output: A follow-up assessment plan grounded in hospitalization evidence, focusing on recovery trajectory, medication tolerance, renal/cardiac monitoring, steroid instruction clarity, function, cognition, support, and escalation needs.

Failure Design

Key Trap	Remediation
Invented follow-up facts	Use the +7 date as a review anchor only; do not create new symptoms, labs, vitals, visits, recovery, or deterioration.
Medication monitoring gap	Identify renal function, potassium, blood pressure symptoms, volume status, intake, diabetes therapy, and HF/CAD sequencing as follow-up priorities.
Prednisone plan vagueness	Clarify what should be checked about prednisone instructions and source consistency without assigning a fixed dose in the prompt or inventing an outcome.
Functional recovery assumption	Compare follow-up needs to baseline function, cognition, medication-management ability, and family support.

Task-level files: FI-T05. World-level files only otherwise.

Task 6: Patient-Safety / Readmission-Risk Review

Workflow: Physician-facing quality or transition review at the +30 discharge anchor.

Draft Prompt

Please review Mr. Merrow's hospitalization and discharge-planning record for patient-safety and readmission-risk signals that were visible before discharge, without assuming whether a readmission or adverse event occurred.

Expected Output: A retrospective risk review that identifies pre-discharge signals around medication sequencing, functional/cognitive vulnerability, family support, consultant synthesis, source hierarchy, and over-trust of the discharge-facing plan.

Failure Design

Key Trap	Remediation
Outcome invention	Use the +30 date as a review anchor only; do not create readmission, non-readmission, adverse event, recovery, or blame narrative.
Visible-source overtrust	Recognize that a reassuring discharge-facing plan may leave unresolved medication, therapy, education, support, and attending-review items.
Distributed evidence miss	Pull safety signals from nursing, PT/OT, family, consultants, MAR/trends, and case-management sources rather than physician summaries alone.
Friction collapse	Preserve defensible positions from family, primary team, Cardiology, Nephrology, and Endocrinology while identifying safety implications.

Task-level files: FI-T06. World-level files only otherwise.

3. World File Plan

Design note: world-level files are available to all tasks. Task-level files provide request context for specific tasks. Supplementary files add realistic background or logistics and should not carry sole critical evidence. Total file count: 22 essential world-level + 7 essential task-level + 4 supplementary = 33 unique files.

3.1 Essential Files (World-Level)

#	ID	Filename.type	Date	Reference File Origin	Description	Pearls, Traps, and Friction
1	FI-W01	FI-W01_ed-triage-initial-intake.docx	05/18/2026 / HD1	Writer-produced file; source role: ED triage nursing	Initial intake, symptoms, family concern, near-fall, possible urinary symptoms, altered baseline.	Seeds infection frame, functional concern, and family/team perspective.
2	FI-W02	FI-W02_ed-provider-assessment.docx	05/18/2026 / HD1	Writer-produced file; source role: Emergency Medicine provider	ED assessment and early sepsis-oriented stabilization logic.	Supports reasonable initial infection framing without closing the case.
3	FI-W03	FI-W03_admission-history-and-physical.docx	05/18/2026 / HD1	Writer-produced file; source role: Hospitalist service	Admission H&P with working problem list, baseline history, comorbidity context, and initial medication concerns.	Seeds prednisone, HF/AKI, and sepsis-anchoring problems.
4	FI-W04	FI-W04_initial-medication-reconciliation-note.docx	05/18/2026 / HD1	Writer-produced file; source role: Pharmacy med rec with patient/family collateral	Initial medication claims, family/patient history, and medication uncertainty.	Key medication and prednisone source channel; not a final discharge med list.
5	FI-W05	FI-W05_pharmacy-refill-history-report.docx	Pre-admission provenance reviewed HD1	Writer-produced file; source role: Pharmacy history	External refill provenance for chronic cardiac, diabetes, prednisone, supplement, and supportive medications.	Supports medication history without outranking rheumatology for prednisone intent.
6	FI-W06	FI-W06_outpatient-rheumatology-prednisone-provenance.docx	Pre-admission provenance available by HD4	Writer-produced file; source role: Outpatient rheumatology	Highest-authority PMR and prednisone taper provenance plus chronic steroid-exposure context.	Central for prednisone reconstruction; availability timing matters.
7	FI-W07	FI-W07_primary-care-outpatient-baseline-summary.docx	Pre-admission provenance during hospitalization	Writer-produced file; source role: Primary care documentation	Baseline function, chronic disease context, outpatient medication continuity, and follow-up context.	Supports baseline comparison, discharge readiness, and source hierarchy.
8	FI-W08	FI-W08_hd1-hd2-hospitalist-progress-documentation.docx	05/18-05/19/2026 / HD1-HD2	Writer-produced file; source role: Hospitalist service	Early response, stabilization, evolving diagnostic frame, and early medication-safety reasoning.	Shows improvement while preserving medication and anchoring risks.
9	FI-W09	FI-W09_hd3-hospitalist-progress-documentation.docx	05/20/2026 / HD3	Writer-produced file; source role: Hospitalist service	Transition from acute stabilization toward functional/cognitive and discharge-readiness concerns.	Moves attention from infection response to function and safety.
10	FI-W10	FI-W10_hd4-hospitalist-progress-documentation.docx	05/21/2026 / HD4	Writer-produced file; source role: Hospitalist service	Consultant tensions, steroid-history inconsistency, and primary-team synthesis burden.	Supports all three frictions without choosing a winner.
11	FI-W11	FI-W11_hd5-hd6-hospitalist-discharge-planning-progress-documentation.docx	05/22-05/23/2026 before 18:00	Writer-produced file; source role: Hospitalist service	Medical improvement with unresolved discharge, medication, steroid, and family concerns.	Establishes visible improvement but incomplete readiness.
12	FI-W12	FI-W12_objective-renal-infection-hemodynamic-trend-summary-source.docx	HD1-HD6 through 05/23/2026 18:00	Writer-produced data source; source role: laboratory/vital trends	Objective renal, infection, potassium, hemodynamic, intake, and glucose trend substrate.	Supports reassessment but does not settle restart or disposition decisions alone.
13	FI-W13	FI-W13_medication-administration-inpatient-medication-action-source.docx	HD1-HD6 through 05/23/2026 18:00	Writer-produced data source; source role: MAR / inpatient medication actions	Medication holds, continuations, reassessments, and inpatient-only actions.	Not a final discharge medication plan.
14	FI-W14	FI-W14_nephrology-consultation-documentation.docx	HD2-HD6, strongest HD4-HD6	Writer-produced file; source role: Nephrology	AKI-on-CKD recovery, volume, potassium, blood pressure, and renal medication safety.	Defensible renal-safety side of cardiorenal friction.
15	FI-W15	FI-W15_cardiology-consultation-documentation.docx	HD2-HD6, strongest HD4-HD6	Writer-produced file; source role: Cardiology	HFrEF/CAD protective therapy, GDMT restart concern, and decompensation risk.	Defensible cardiovascular-protection side of cardiorenal friction.
16	FI-W16	FI-W16_endocrinology-consultation-documentation.docx	HD2-HD6, strongest HD4-HD6	Writer-produced file; source role: Endocrinology	Steroid/adrenal risk interpretation and safe prednisone/taper planning.	Relevant steroid-risk perspective without becoming a single-diagnosis reveal.
17	FI-W17	FI-W17_bedside-nursing-observation-notes-flowsheet-summary.docx	HD1-HD6 through 05/23/2026 18:00	Writer-produced file; source role: Bedside nursing	Cognition, intake, ambulation tolerance, weakness, family concerns, and practical safety observations.	Important functional/cognitive evidence can be easy to miss.
18	FI-W18	FI-W18_physical-therapy-assessment.docx	HD3-HD6 through 05/23/2026 18:00	Writer-produced file; source role: Physical Therapy	Mobility, transfer safety, endurance, assistance needs, and discharge functional recommendations.	Key functional evidence for discharge planning.
19	FI-W19	FI-W19_occupational-therapy-assessment.docx	HD3-HD6 through 05/23/2026 18:00	Writer-produced file; source role: Occupational Therapy	ADL safety, medication-management ability, cognitive/functional home-task concerns.	Supports function and medication-management safety.
20	FI-W20	FI-W20_family-communication-care-conference-	HD3-HD6 through 05/23/2026 18:00	Writer-produced file; source role: documented Mara Mellow report	Family baseline knowledge, home safety, medication-management	Supports family/team friction and lower-authority prednisone

#	ID	Filename.type	Date	Reference File Origin	Description	Pearls, Traps, and Friction
21	FI-W21	documentation.docx FI-W21_case-management-social-work-discharge-planning-note.docx	HD5-HD6 through 05/23/2026 18:00	Writer-produced file; source role: Case Management / Social Work	concerns, and discharge concern. Services, support, caregiver capacity, equipment, transportation, or placement considerations.	collateral. Supports care coordination and transition logistics; not sole safety evidence.
22	FI-W22	FI-W22_discharge-facing-plan-snapshot-before-world-close.docx	05/23/2026 before 18:00 / HD6	Writer-produced file; source role: Hospitalist / interdisciplinary planning	Visible discharge-facing plan before world close.	Useful and reassuring but incomplete; requires reconciliation with the broader chart.

3.2 Essential Files (Task-Level)

#	ID	Filename.type	Date	Reference File Origin	Description	Pearls, Traps, and Friction
1	FI-T01	FI-T01_discharge-medication-reconciliation-request-context.docx	05/24/2026	Writer-produced task context; requester: hospitalist/pharmacy/discharge clinician	Frames medication-safety review using the chart without creating the final medication list.	Supports Task 1 and medication-source reasoning.
2	FI-T02	FI-T02_discharge-summary-drafting-request-context.docx	05/24/2026	Writer-produced task context; requester: attending hospitalist	Frames discharge-summary drafting without copying the early diagnosis frame forward.	Supports Task 2 and course synthesis.
3	FI-T03	FI-T03_discharge-readiness-care-coordination-request-context.docx	05/24/2026	Writer-produced task context; requester: hospital team/case management	Frames discharge-readiness and care-coordination assessment.	Supports Task 3 and family/team disposition reasoning.
4	FI-T04	FI-T04_consultant-synthesis-interdisciplinary-care-plan-request-context.docx	05/24/2026	Writer-produced task context; requester: hospitalist-led interdisciplinary team	Frames consultant synthesis and interdisciplinary care planning.	Supports Task 4 and all three frictions.
5	FI-T05	FI-T05_early-post-discharge-follow-up-assessment-request-context.docx	05/31/2026	Writer-produced task context; requester: primary care or transition team	Frames +7 follow-up priorities based on hospitalization evidence only.	Supports Task 5; no new +7 clinical findings.
6	FI-T06	FI-T06_patient-safety-readmission-risk-review-request-context.docx	06/23/2026	Writer-produced task context; requester: quality/safety/transition team	Frames +30 retrospective safety review based on pre-discharge evidence.	Supports Task 6; no invented readmission or outcome.
7	FI-T07	FI-T07_medication-safety-handoff-task-context-addendum.docx	05/24/2026	Writer-produced task context addendum; source role: pharmacy/discharge team	Narrow medication-safety addendum for holds, restarts, taper uncertainty, and home management.	Supports Task 1 only; not a seventh task.

3.3 Supplementary Files

#	ID	Filename.type	Date	Reference File Origin	Description	Pearls, Traps, and Friction
1	FI-S01	FI-S01_remote-pci-coronary-stent-provenance-summary.docx	Pre-admission provenance	Writer-produced supplementary provenance; source role: cardiology/PCP history	Remote PCI/stent background and chronic cardiovascular prevention context.	Background only; indirect medication-reconciliation support.
2	FI-S02	FI-S02_remote-sleep-study-osa-provenance-summary.docx	Pre-admission provenance	Writer-produced supplementary provenance; source role: PCP/sleep history	Remote sleep-study background supporting OSA as chronic reserve context.	Background only; does not explain acute decline.
3	FI-S03	FI-S03_home-support-equipment-logistics-reference.docx	HD5-HD6 through 05/23/2026 18:00	Writer-produced supplementary administrative file; source role: Case Management/Social Work	Home support, equipment, and logistics texture.	Secondary discharge-planning context; not a final service authorization.
4	FI-S04	FI-S04_problem-list-past-history-snapshot.docx	HD1 or pre-admission import	Writer-produced supplementary chart snapshot; source role: problem list/past history	Problem-list and past-history background texture.	Low-authority context only; does not outrank medication or rheumatology sources.

Self-containment principle

Every correct task response must be traceable to the prompt, its task-level context files, and the world-level file set. Supplementary files can add realism and background, but no task should depend on a supplementary file as the sole source of critical evidence.

4. World Summary

Korvin Merrow tests whether a clinician can manage a familiar but messy inpatient transition: an older medically complex patient improves after admission, but the work is not finished simply because the first diagnosis was reasonable and the patient looks better. The chart asks the clinician to separate acute stabilization from discharge readiness.

The case requires synthesis across time, source reliability, consultant recommendations, medication actions, objective trends, bedside functional evidence, family baseline knowledge, and discharge logistics. A strong response should preserve uncertainty where the chart is uncertain, avoid making prednisone or infection the single answer, and explain how discharge planning can be clinically defensible while still operationally risky.

The six tasks use the same world from different physician-facing angles: medication reconciliation, discharge summary, care coordination, interdisciplinary synthesis, early follow-up planning, and later safety review. The common thread is careful clinical judgment under real-world documentation noise.